



University of Al-Ameed
College of Medicine

**Department of
Internal Medicine**



History taking
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Objectives

History taking

- **Knowledge:**
 - Understand the basic components of history taking (chief complaint, history of present illness, etc.).
 - Recall the importance of each part of the history in aiding diagnosis.
- **Comprehension:**
 - Explain the significance of patient history in confirming diagnoses, as over 60% of diagnoses are made from history alone.
- **Application:**
 - Apply effective communication skills during patient interviews.
 - Use appropriate techniques in different clinical settings (outpatient, inpatient, emergency).
- **Analysis:**
 - Analyse symptoms to differentiate between normal and abnormal findings.
 - Evaluate the patient's responses to identify potential diagnoses.
- **Synthesis:**
 - Integrate information from the history to formulate a provisional diagnosis.
 - Develop a comprehensive patient profile that includes social, medical, and family histories.
- **Evaluation:**
 - Assess the effectiveness of the history taking process.
 - Reflect on the accuracy of provisional diagnoses based on the gathered history.

- Why do we take history from the patient?
- To confirm diagnosis.
- More than 60% of the patients a diagnosis is made from the history

How do we take history?

What do we need ?

We need :

- ❖ Available patients.
- ❖ • Appropriate setting.
- ❖ • Comfortable place & time as possible.
- ❖ • Patience and understanding.
- ❖ • Knowledge.
- ❖ • Good communication skills.
- ❖ • Experience.

SET UP OF HISTORY TAKING

- 1. In the outpatient clinic : short but effective focused & informative history
- 2. In the inpatient clinic: long classical history
- 3. In the emergency room: Chief complaint followed by Checking the Vital Signs and than the secondary survey.



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COMPONENTS OF THE HISTORY

- The present complaint
- The history of the present complaint
- Remaining questions of abnormal system
- Review of systems
- Past medical history
 - ⊕ Past surgical history
 - ⊕ Drug history
 - ⊕ Immunizations
 - ⊕ Family history
 - ⊕ Social history & habits

DON'T FORGET TO :

- ❖ INTRODUCE YOURSELF TO THE PATIENT AND EXPLAIN TO HIM OR HER WHAT YOU ARE GOING TO DO.
- ❖ ASK THEM WHAT THEY LIKE TO BE CALLED (e.g. SIR, MADAM, DOCTOR, PROFESSOR ,...).
- ❖ GET A CHAPERON WHEN YOU INTERVIEW A FEMALE PATIENT.
- ❖ BE A GOOD LISTENER, AT THE SAME TIME TAKE CONTROL OF THE SESSION.
- ❖ USE A LOT OF BODY LANGUAGE IN YOUR COMMUNICATION WITH THE PATIENT.

FIRST : IDENTIFICATION DATA

- ❖ Name
- ❖ Age
- ❖ Sex
- ❖ Marital status
- ❖ Occupation
- ❖ Address
- ❖ Date of interview

1-PRESENT (CHIEF) COMPLAINT

Definition : The main complaint that brought the patient to medical attention.

- ❖ In patient's own words (try to avoid medical terms , use simple English ... ex : diarrhea to be replaced by watery stool)
- ❖ With duration (in weeks, days, hours , minutes, seconds).
- ❖ If many complaints given, choose the most bothersome one .
- ❖ “What are you complaining of?”
- ❖ “What is the problem?”
- ❖ “What is the matter?”

2-HISTORY OF THE PRESENT COMPLAINT

EXAMPLE: ABDOMINAL PAIN

- Site
 - ⊕ Radiation
- Time and mode of onset
 - ⊕ Progression/end
- Nature
 - ⊕ Relieving factors
- Duration
 - ⊕ Exacerbating factors
- Severity
 - ⊕ Cause

2.2 Characteristics of pain (SOCRATES)

Site

- Somatic pain, often well localised, e.g. sprained ankle
- Visceral pain, more diffuse, e.g. angina pectoris

Onset

- Speed of onset and any associated circumstances

Character

- Described by adjectives, e.g. sharp/dull, burning/tingling, boring/stabbing, crushing/tugging, preferably using the patient's own description rather than offering suggestions

Radiation

- Through local extension
- Referred by a shared neuronal pathway to a distant unaffected site, e.g. diaphragmatic pain at the shoulder tip via the phrenic nerve (C₅, C₆)

Associated symptoms

- Visual aura accompanying migraine with aura
- Numbness in the leg with back pain suggesting nerve root irritation

Timing (duration, course, pattern)

- Since onset
- Episodic or continuous:
 - If episodic, duration and frequency of attacks
 - If continuous, any changes in severity

Exacerbating and relieving factors

- Circumstances in which pain is provoked or exacerbated, e.g. eating
- Specific activities or postures, and any avoidance measures that have been taken to prevent onset
- Effects of specific activities or postures, including effects of medication and alternative medical approaches

Severity

- Difficult to assess, as so subjective
- Sometimes helpful to compare with other common pains, e.g. toothache
- Variation by day or night, during the week or month, e.g. relating to the menstrual cycle

3-REMAINING QUESTIONS OF ABNORMAL SYSTEM

- ❖ Try to focus on 1 system of blame , e.g. GIT, CVS, CNS.
- ❖ Ask other symptoms that are associated , e.g. if abdominal pain ask about nausea , vomiting, change in bowel habit...

IS IT TIME TO MAKE A PROVISIONAL DIAGNOSIS?

- The identification of the nature of an illness or other problem by examination of the symptoms.

What are the types of diagnosis :

- ❖ Provisional diagnosis : primary one, based on available information.
- ❖ Differential Diagnosis.
- ❖ Final diagnosis : confirmed by lab tests or natural history of the disease.

DIFFERENTIAL DIAGNOSIS OR WORKING DIAGNOSIS

- Most likely why?
- Less likely why?
- Least likely why?

DIAGNOSIS

Any diagnosis consists of:

- Anatomical part + Pathological part

Examples:

- ❖ Breast cancer
- ❖ Peptic ulcer
- ❖ Fracture femur
- ❖ Irritable Bowel Syndrome
- ❖ Ischemic Heart Disease

4-REVIEW OF SYSTEMS

- ❖ The Gastro-intestinal system
- ❖ The Respiratory system
- ❖ The Cardiovascular system
- ❖ The Urogenital system
- ❖ Endocrine
- ❖ The Nervous system
- ❖ The Musculoskeletal system

2.3 Questions to ask about common symptoms

System	Question
Cardiovascular	Do you ever have chest pain or tightness? Do you ever wake up during the night feeling short of breath? Have you ever noticed your heart racing or thumping?
Respiratory	Are you ever short of breath? Have you had a cough? If so, do you cough anything up? What colour is your phlegm? Have you ever coughed up blood?
Gastrointestinal	Are you troubled by indigestion or heartburn? Have you noticed any change in your bowel habit recently? Have you ever seen any blood or slime in your stools?
Genitourinary	Do you ever have pain or difficulty passing urine? Do you have to get up at night to pass urine? If so, how often? Have you noticed any dribbling at the end of passing urine? Have your periods been quite regular?
Musculoskeletal	Do you have any pain, stiffness or swelling in your joints? Do you have any difficulty walking or dressing?
Endocrine	Do you tend to feel the heat or cold more than you used to? Have you been feeling thirstier or drinking more than usual?
Neurological	Have you ever had any fits, faints or blackouts? Have you noticed any numbness, weakness or clumsiness in your arms or legs?

GASTRO-INTESTINAL SYSTEM

- Appetite
 - Diet
 - Weight
 - Teeth and taste
 - Swallowing
 - Regurgitation
 - Fatulence
- ⊕ Heartburn
 - ⊕ Vomiting
 - ⊕ Haematemesis
 - ⊕ Abdominal PAIN
 - ⊕ Abdominal distension
 - ⊕ Defecation
 - ⊕ Change of color of skin

THE RESPIRATORY SYSTEM

- ❖ Cough
- ❖ Sputum
- ❖ Hemoptysis
- ❖ Dyspnea
- ❖ Orthopnea
- ❖ Chest pain

THE CARDIOVASCULAR SYSTEM

- ❖ CHEST PAIN
- ❖ Dyspnea
- ❖ Orthopnea
- ❖ Palpitations
- ❖ Cough and sputum
- ❖ Dizziness and headache
- ❖ Ankle swelling
- ❖ Peripheral vascular symptoms

THE UROGENITAL SYSTEM

- Pain
- Oedema
- Thirst
- Micturition
- Urine

- ⊕ Scrotum and urethra
- ⊕ Menstruation
- ⊕ Pregnancies
- ⊕ Breasts
- ⊕ Secondary sex characteristics

THE NERVOUS SYSTEM



- ❖ Mental state
- ❖ Conscious level
- ❖ Fits & abnormal movements
- ❖ TIAS= transient ischemic attacks
- ❖ Loss of sensations
- ❖ Paraesthesia (pins and needles)
- ❖ Speech difficulties
- ❖ Unsteadiness

THE MUSCULOSKELETAL SYSTEM

- ❖ **Pain**
- ❖ **Swelling**
- ❖ **Limitation of movements of any joint**

5-PAST MEDICAL HISTORY

- ❖ Any hospitalization
- ❖ TB = Tuberculosis
- ❖ Diabetes mellitus
- ❖ Hypertension
- ❖ Asthma
- ❖ Rheumatic fever
- ❖ Contact with patients with hepatitis or aids

6-PAST SURGICAL HISTORY

- ❖ Previous operations
- ❖ Blood transfusion
- ❖ Any complications with anesthesia
- ❖ Bleeding tendencies

7-DRUG HISTORY

- ❖ Steroids
- ❖ None steroidal anti-inflammatory drugs
- ❖ Insulin and other oral hypoglycemic drugs
- ❖ Antihypertensive drugs
- ❖ Hormone replacement therapy

8-IMMUNIZATIONS

- ❖ DPT = diphtheria, pertussis, tetanus
- ❖ Measles
- ❖ Mumps
- ❖ Rubella
- ❖ Poliomyelitis
- ❖ TB
- ❖ Smallpox
- ❖ Typhoid

9-FAMILY HISTORY

- ❖ Health and age or cause of death of patient's parents ,brothers and sisters.
- ❖ Presence of common illnesses (DM, HT , Asthma) in the family.
- ❖ Consanguinity

10-SOCIAL HISTORY & HABITS

- ❖ Marital status
- ❖ Hazards of occupation
- ❖ Social status- type of residence
- ❖ Travel abroad-dates
- ❖ Smoking
- ❖ Alcohol Drink.
- ❖ Any unusual?

SUMMARY

- ❖ Patient's name, age and sex.
- ❖ Complaint and the most important positive characteristics of his/her complaint
- ❖ The most important negative features of his complaint.